- Extensive activation of clotting factors together with immunity.

ii. Blood volume: During delivery and soon after 1/5 of the volume is reduced due to
  - Blood loss in normal delivery – 400 ml and in caesarean section – 1000 ml
  - Obliteration of loss – resistance uteroplacental circulation
  - Maternal vascular bed decreases 10 – 15 %

iii. Hematopoiesis:
  - Leukocytosis – 25000/mm^3 during labour and puerperium.
  - Bone marrow hyperactivity present
  - Iron decreases,becomes normal by 2nd week.

Respiratory Changes

- Residual volume increase.
- Vital capacity and inspiratory capacity decrease.
- Post partum decreased progesterone and decreased respiration increase PCO2.

Menstruation

- Non-lactating: starts at about 12 weeks.
- Lactating: starts after 6 months
- 1st cycle: anovulatory or inadequate

Lactation

- Prolactin is an obligatory hormone which increases as pregnancy advances.
- Prolactin induced secretory activity is placental sex steroid.
- After delivery decreased sex steroid of lactation.
- Milk ejection by myoepithelial cells contraction. Milk propelled along with the ducts into lactiferous sinus beneath the areola.
- Neurohormonal reflex: afferent pathway to hypothalamus are stimulated by suckling through 4th and 6thintercostals nerve. Oxytocin is released by stimulation of auditory and visual efferent limb.
- Maintenance: suckling and emptying of duct and alveoli.

Complications Of Puerperium

- Genital tract infection
- Urinary tract infection
- Wound infection
- Mastitis
- Thromboembolism
- Incontinence/ urinary retention
- Anal sphincter dysfunction
- Episiotomy wound gaping

Postnatal Care

Mother:

a) **Rest and sleep:** Adequate rest and sleep are vital for maternal health, lactation and baby care.

b) **Diet:** A balanced diet contains high protein, carbohydrate, fats, minerals and vitamins. Additional calories are required for lactation. (500 calories/day)

c) **Ambulation:** Ambulation should be encouraged as soon as possible, 6 hours after normal delivery and 24 hours after a caesarean delivery. This helps preventing thrombo-embolism complications, improve venous return and muscle tone.

d) **Bladder:** The postnatal mother usually passes urine within 6 hours. In case of any difficulties, she should be encouraged to do so, by ambulation, plenty of oral fluids, opening the water tap while she is trying to pass urine, pour hot and cold water over vulva.

e) **Bowel:** Constipation is common due to painful perineum. Plenty of fluids, green leafy vegetables and roughage in diet will help.

Unit_7_Midwifery_Gen_Nur.indd 166 09-03-2020 14:51:13